

City General Hospital

Department of Pulmonology & Respiratory Medicine

Patient Name:	Priya Sharma	MRN:	MGH-2024-13377
Age / Sex:	34 years / Female	Date:	05-Mar-2026
DOB:	17-Jul-1991	Physician:	Dr. Arjun Menon, MD (Pulmonology)

INPATIENT ADMISSION REPORT

Chief Complaint: High-grade fever, productive cough with yellow-green sputum, and right-sided chest pain for 5 days.

History of Present Illness: Ms. Priya Sharma is a 34-year-old non-smoker who presents with a 5-day history of high-grade fever (up to 40.1°C), productive cough with purulent yellow-green sputum, right-sided pleuritic chest pain worsening on deep inspiration, and significant fatigue. She reports an upper respiratory tract infection 2 weeks ago that appeared to resolve. No known immunocompromise. No recent travel or sick contacts. Up to date with vaccination. Currently on no regular medications.

VITAL SIGNS

Temperature:	39.8 °C (Febrile)	Heart Rate:	108 bpm (Tachycardia)
Blood Pressure:	118/74 mmHg	SpO2:	92% on room air
Respiratory Rate:	26 breaths/min	CRB-65 Score:	2 (Moderate Severity)

CHEST X-RAY FINDINGS

PA Chest X-Ray — 05-Mar-2026: Right lower lobe consolidation with air bronchograms clearly visible. Blunting of the right costophrenic angle suggestive of a small right pleural effusion. Left lung clear. No pneumothorax. Cardiac silhouette normal. **Impression: Right lower lobe consolidation — Community-Acquired Pneumonia.**

LABORATORY RESULTS

Test	Result	Normal Range	Unit	Flag
WBC Count	18,400	4000 – 11000	cells/μL	HIGH ↑↑
Neutrophils	82	40 – 70	%	HIGH ↑
CRP	142	< 5.0	mg/L	HIGH ↑↑
Procalcitonin (PCT)	2.8	< 0.1	ng/mL	HIGH ↑↑
ESR	88	< 20	mm/hr	HIGH ↑
Haemoglobin	12.1	12.0 – 16.0	g/dL	Normal
Lactate	2.1	0.5 – 1.6	mmol/L	HIGH ↑
Sodium	134	135 – 145	mEq/L	LOW ↓
Creatinine	0.9	0.5 – 1.1	mg/dL	Normal
Blood Culture	Pending	Negative		Pending
Sputum Culture	Gram-positive cocci in	Negative		Abnormal

CLINICAL NOTES

Clinical, radiological, and laboratory findings are consistent with moderate-severity Community-Acquired Pneumonia (CAP) affecting the right lower lobe. The markedly elevated WBC (18,400), CRP (142 mg/L), and PCT (2.8 ng/mL) indicate significant bacterial infection. Sputum showing gram-positive cocci in clusters raises suspicion for *Streptococcus pneumoniae* or *Staphylococcus aureus*. Mildly elevated lactate (2.1 mmol/L) warrants monitoring for early sepsis. CRB-65 score of 2 indicates moderate severity — hospital admission and parenteral antibiotics indicated. Initiated: IV Amoxicillin-Clavulanate 1.2g TDS + Azithromycin 500 mg OD. Awaiting blood and sputum culture sensitivity. Physiotherapy for chest clearance initiated.

PRELIMINARY DIAGNOSIS

Community-Acquired Pneumonia (CAP) — Right Lower Lobe, Moderate Severity (CRB-65: 2). Suspected bacterial aetiology. Small right pleural effusion. Monitor for sepsis.